

1. Administrative & Study Identification

Full Study Title: A Phase II, Multicentre, Open-label, Single-arm Study of AZD0901 Monotherapy in Second-or Later-Lines Adult Participants With Advanced/Metastatic Gastric or Gastroesophageal Junction Adenocarcinoma Expressing Claudin18.2 **Short Title/**
Acronym: GAMBIT **Protocol Number:** AZ-RU-00007 **NCT Number:** NCT07143604 **Sponsor Name:** AstraZeneca **Investigational Product:** AZD0901 (Sonesitatus Vedotin) **Phase of Development:** Phase II
Medical Monitor: AstraZeneca Clinical Study Information Center

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To measure the efficacy and safety of AZD0901 monotherapy as a second-line or later (2L+) treatment for participants with advanced or metastatic gastric or gastroesophageal junction (GEJ) adenocarcinoma expressing CLDN18.2. **Secondary Objectives:** To assess the Time to Response (TTR) and Incidence of Treatment-Emergent Adverse Events (TEAEs) to establish tolerability.

2.2 Background & Rationale To address the unmet need for targeted treatments in patients with advanced or metastatic gastric or GEJ adenocarcinoma who have experienced disease progression after at least one prior systemic regimen (including a fluoropyrimidine and a platinum). AZD0901 (sonesitatus vedotin) is an innovative Claudin18.2-specific antibody-drug conjugate (ADC) aimed at evaluating target-specific tumor responses in this population.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label (None) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 33 participants (Actual Enrollment) **Number of Sites:** 13 locations in the Russian Federation **Estimated Study Start:** 29-Aug-2025 **Estimated Primary Completion:** 28-Sep-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histologically confirmed unresectable, locally advanced or metastatic adenocarcinoma of gastric, GEJ, or distal esophagus, with positive CLDN18.2 expression. 2. Disease progression on or after at least one prior regimen for advanced

or metastatic disease, which included a fluoropyrimidine and a platinum. 3. Must have at least one measurable lesion assessed by the Investigator based on RECIST 1.1, an ECOG performance status of 0 or 1, and a minimum life expectancy of $\geq$ 12 weeks.

4.2 Exclusion Criteria 1. Participants with known HER2-positive status as defined as IHC 3+ or IHC 2+/ISH+. 2. Participant has significant or unstable gastric bleeding and/or untreated gastric ulcers. 3. CNS metastases or CNS pathology, clinically significant corneal disease (e.g., active keratitis), or persistent toxicities (CTCAE Grade $\geq$ 2) caused by previous anticancer therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: AZD0901 monotherapy administered once every 3 weeks (Q3W). **Route of Administration:** Intravenous (IV). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, withdrawal of consent, or study completion (Estimated completion 28-Sep-2027).

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications required for managing routine toxicities. **Prohibited:** Concurrent administration of other systemic anticancer treatments or investigational therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, CLDN18.2 & HER2 Testing, Baseline Imaging (RECIST 1.1) Baseline Day 0 Final Eligibility Confirmation, First IV Dose Interim Every 3 Weeks (Q3W) AZD0901 Administration, Safety Labs, Efficacy Assessment (Tumor Imaging) End of Study Follow-up Final Vital Signs, Treatment Discontinuation visit, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Objective tumor response indicating efficacy. **Measurement Method:** Evaluated by the investigator based on Response Evaluation Criteria in Solid Tumors (RECIST 1.1).

7.2 Secondary & Exploratory Endpoints * **Time to Response (TTR):** Defined as the time from the start of the treatment to the first documented objective tumor response observed for participants who achieved Complete Response (CR) or Partial Response (PR) per RECIST 1.1. * **Incidence of TEAEs:** Monitoring of treatment-emergent adverse events for safety and tolerability profiling.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinical visits every 3 weeks (Q3W), graded utilizing CTCAE criteria. **Serious Adverse Events (SAEs):** Investigated and reported within the standard 24-hour timeline required for oncology Phase II trials. **Data Monitoring Committee (DMC):** Internal safety monitoring mechanisms managed by AstraZeneca.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All dosed participants with measurable disease at baseline who have a response, regardless of whether the participant withdraws from therapy, receives another anticancer treatment, or clinically progresses prior to PD. **Sample Size Justification:** $N=33$ appropriately powered for a Phase II single-arm proof-of-concept and safety assessment. **Handling of Missing Data:** Standard oncology data imputation, treating early withdrawals as non-responders for the primary efficacy analysis where applicable.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine remote and onsite monitoring visits to verify dosing, safety records, and RECIST evaluations. **Audit Trail:** Robust eCRF locking mechanisms and source data verification processes.

11. Study Identifiers & Governance

Primary ID: AZ-RU-00007 **Registry Number:** NCT07143604 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** GAMBIT **Official Regulatory Title:** A Phase II, Multicentre, Open-label, Single-arm Study of AZD0901 Monotherapy in Second-or Later-Lines Adult Participants With Advanced/Metastatic Gastric or Gastroesophageal Junction Adenocarcinoma Expressing Claudin18.2

12. Clinical Framework (Gastric Cancer Specific)

Primary Condition: Advanced or Metastatic Gastric or GEJ Adenocarcinoma **Diagnosis Threshold:** Positive Claudin18.2 (CLDN18.2) expression and negative HER2 status **Medical Methodology:** Claudin18.2-directed Antibody-Drug Conjugate (ADC) therapy **Optimization Target:** Objective tumor response and Time to Response (TTR)

13. Study Procedures & Methodology

Management Pathway: Standard 2L+ oncology clinical pathway.
Education Protocol (HCP): Site initiation visits and RECIST 1.1 response evaluation training. **Education Protocol (Patient):** Informed consent process, toxicity expectations (e.g., corneal disease tracking), and self-monitoring guidance. **Quality Audit Mechanism:** Independent imaging review capabilities and AE reporting validation.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up through expected study completion (estimated September 28, 2027) **Onsite Visit Cadence:** Every 3 Weeks (Q3W) concurrent with IV dosing **Remote Monitoring Frequency:** Standard safety check-ins as clinically indicated between dosing cycles **Checklist Review Interval:** Baseline and at the start of each Q3W cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				